First-week packet actions

A 20-minute huddle for nonclinical caseworkers | Duluth, Minnesota and Superior, Wisconsin teams

Purpose Turn overseas medical packet information into safe scheduling and escalation actions. Keep the record’s words intact; do not turn labels into diagnoses.	**Your role** Observe, record, schedule, safeguard information, and escalate. You do **not** diagnose, decide treatment, determine infectiousness, or conduct clinical triage.

The priority order: respond before you read

Use this sequence every time. It is action routing, not diagnosis.

ACT FIRST	Caseworker action	Packet does / does not tell you
1 Immediate emergency	Call emergency services for severe breathing difficulty, unresponsiveness, or another immediate medical emergency. For an immediate threat of self-harm or harm to others, use emergency services. Then notify Selma when safe.	Current presentation or immediate safety concern changes the response regardless of what the packet says. A historical behavioral notation alone is not a present emergency.
2 Rare exception	Class A TB with a waiver: **same-day** coordinate with Selma and the appropriate public-health role. Keep it out of the ordinary appointment queue.	Do not impose isolation, cancel ordinary transport automatically, notify housing contacts, or make exposure notices. Do not decide infectiousness.
3 Prioritized coordination	For TB follow-up notations or other findings flagged for follow-up, promptly coordinate with Selma / receiving clinical-public-health role before ordinary screening.	The letter alone is not a severity scale. Use the exact notation, packet instructions, current facts, and receiving pathway.
4 Ordinary scheduling	Arrange ordinary domestic screening in 30–90 days after arrival. Higher-priority findings override this ordinary queue.	Screening is a receiving-clinic service, not caseworker assessment.

Open the packet without changing it

The original record is evidence for receiving professionals. Your work is a narrow, documented handoff.

1	**Match first** Before acting, verify the packet belongs to the correct person: match name plus date of birth or another packet identifier. Do not copy more identifiers than necessary.
2	**Preserve the original** Do not alter, annotate over, discard, or separate original overseas medical records. Securely store them and use secure transmission, never personal email, open messaging, or unattended copies.
3	**Read and relay exactly** Record the classification and annotation exactly as written; attach or securely relay the record when needed. Do not paraphrase it into a diagnosis.
4	**Exception? Do not reconstruct** Missing, damaged, mismatched, or unreadable packet: secure it and escalate to Selma / the appropriate receiving role. Do not guess or reconstruct contents.

What Class A and Class B mean - and do not mean

These are federal overseas classifications. Their meaning does not change at the Minnesota-Wisconsin state line.

Class A	Class B
An **overseas immigration medical classification** associated with an inadmissible condition unless an applicable waiver or resolution permits travel. Arrival may occur with an authorized waiver or other documented clearance pathway. It is **not** a universal statement of present infectiousness.	A significant health condition, finding, or departure from normal health that warrants follow-up. It is **not** itself an inadmissible Class A condition and is **not**, by itself, a diagnosis of active or contagious disease.

TB notations: quote, route, do not diagnose

All TB follow-up rows are prioritized coordination ahead of ordinary screening. They are not caseworker symptom screening rules.

Exact packet notation	What it supports	Caseworker action
Class B0 pulmonary TB	TB disease was diagnosed overseas and treatment was completed under the applicable overseas process before departure.	Promptly coordinate domestic follow-up with Selma / receiving clinical-public-health role; do not recast as untreated active TB.
Class B1 pulmonary TB	Findings or history require domestic follow-up, without an overseas diagnosis of infectious TB disease at departure.	Prioritized coordination. B1 pulmonary does **not** mean the person has active TB and does **not** authorize you to call the person contagious.
Class B1 extrapulmonary TB	A distinct B1 notation requiring follow-up; it is not synonymous with pulmonary infectiousness.	Prioritized coordination; relay the exact notation.
Class B2 TB	Latent TB infection evaluation - not active TB disease.	Prioritized coordination; do not diagnose.
Class B3 TB	Evaluation as a contact of a known TB case; proves neither infection nor disease.	Prioritized coordination; do not make exposure notices.

Never let a packet delay an emergency

Current symptoms or an immediate safety concern can change the response regardless of what the packet says.

RARE, SEPARATE EXCEPTION	WHAT TO DO
Class A TB with a waiver A person may arrive under an authorized waiver or another documented clearance pathway. The label alone does not establish present infectiousness.	**Same-day** coordinate with Selma and the appropriate public-health role. Do not put it in the ordinary appointment queue. Do not isolate, cancel ordinary transport automatically, warn landlords, or make exposure notifications on your own authority.

Vaccination page: preserve uncertainty

Use the record to support a receiving-clinic handoff, not to complete an immunization assessment yourself.

Read DS-3025 as overseas vaccination documentation
It may show dates or medically relevant reasons a dose was not given (for example, not age appropriate, contraindication, insufficient interval, or unavailable vaccine). It is **not** a complete domestic immunization assessment and does not prove someone is fully vaccinated for every school, employment, or clinical standard.
Blank field = unknown or unrecorded, not proof that a vaccine was never given. Do not fill blank fields from memory, family statements, or assumptions. Preserve the record and arrange clinical review / immunization assessment when documentation is unclear or incomplete; do not conclude compliance.

One workflow, two state routes

The controlled directory supplies the current contacts. Do not invent a deadline, legal mandate, clinic, or public-health contact.

STEP 1 - Select the route	STEP 2 - Use that receiving route
Choose the Minnesota or Wisconsin route by the family's residence or receiving-care location, not airport pickup location alone. The federal meaning of Class A, Class B, B0, B1, B2, and B3 does not change across states.	**Minnesota residence / receiving care:** route to the Minnesota receiving clinic and the appropriate Minnesota public-health role. **Wisconsin residence / receiving care:** route to the Wisconsin receiving clinic and the appropriate Wisconsin public-health role.

Make the handoff, not the diagnosis

The people arranging housing or transport usually need logistics, not health details.

Schedule with the minimum necessary information	Close the loop
Share packet diagnoses or classifications only with the receiving clinical / public-health role that needs them. Do **not** share them with landlords, housing staff, employers, transport providers, or other people who do not need the information. Use a qualified interpreter for health-related communication when needed - not a child or untrained bystander.	Document the appointment or escalation outcome, date/status, destination role, and the **exact packet label relayed**. Do not add a caseworker diagnosis. For a missed, declined, or failed appointment connection: document it, re-engage respectfully, and escalate to Selma / receiving staff; do not let it disappear.

Practice: route the record, not the label

Facilitator prompt: ask for actions, not a diagnosis.

HUDDLE CHECK

Situation: A packet for the correctly matched child shows “Class B1 pulmonary TB.” The family will reside in Wisconsin after a Duluth airport pickup. The child appears comfortable; there is no immediate safety concern. A housing contact asks whether the child has TB. What are your next three actions?

DEBRIEF

1. Preserve and relay the exact notation securely; do not tell housing the child “has TB.” **B1 pulmonary does not mean active TB or authorize calling the child contagious.**
2. Use the Wisconsin residence / receiving-care route, not the Duluth pickup, and prioritize coordination with Selma / Wisconsin receiving clinical-public-health role.
3. Document the exact label and handoff outcome. Emergency services would come first only if the child had an immediate emergency; the packet alone is not a diagnosis.

Clinical owner and escalation point: Selma Rutkowski | Effective September 3 | Controlled release: replaces the interim verbal briefing